

At the present time we have compiled RheumNotes summary of the majority of the conditions listed below. In addition we have RheumNotes on Osteoarthritis, Osteoporosis, Gout, soft tissue rheumatism, FMS, musculoskeletal examination and how to approach the patient with musculoskeletal pain.

Below is a list (generated by AI) of the most common autoimmune diseases treated by rheumatologists globally based upon incidence and prevalence. I intend to compile RheumNotes on each of them as they are inclusive of the most common forms of arthritis and autoimmune disease.

This is a good starting point moving forward.

Top 25 Autoimmune Diseases Treated by Rheumatologists
(ranked primarily by approximate US/global prevalence; incidence noted where data are available).

This list focuses on autoimmune rheumatic diseases (ARDs) or those with prominent rheumatologic manifestations that rheumatologists diagnose and manage (inflammatory arthritides, connective tissue diseases, and vasculitides). Ranking draws from recent US EHR studies, GBD analyses, and epidemiological reviews. Prevalence is the dominant metric for "most common" in these chronic conditions; annual incidence is typically much lower (e.g., 5–50 new cases per 100,000 adults/year for many). Numbers are approximate due to diagnostic/coding variations and underdiagnosis.

Top Tier (Highest Prevalence)

1. **Rheumatoid arthritis (RA)** — US ~2.5M; Global ~17–20M+. Incidence ~20–50/100k py. Most common by far.
2. **Psoriatic arthritis (PsA)** — US ~0.5–1M+; Global ~0.135% prevalence (several million). Incidence ~12/100k py. Strongly linked to psoriasis.
3. **Systemic lupus erythematosus (SLE)** — US ~0.99M; Global hundreds of thousands to millions. Incidence ~5–10/100k py (higher in some groups).
4. **Axial spondyloarthritis (axSpA, incl. ankylosing spondylitis)** — US several hundred thousand+ (broader estimates higher); Global variable (high in HLA-B27 populations). Incidence varies widely.
5. **Sjögren's syndrome** — US ~0.62M; Global low millions in broader estimates. Often overlaps with RA/SLE.

6. **Polymyalgia rheumatica (PMR)** — US ~0.51M; Global higher in Northern European ancestry older adults. Frequently with GCA.

Mid Tier

7. **Systemic sclerosis (scleroderma)** — US ~0.22M; Global rarer (~0.01–0.03%).
8. **Inflammatory myopathies** (dermatomyositis, polymyositis, etc.) — US ~0.1M combined; Global rare (several per 100k).
9. **Giant cell arteritis (GCA, often with PMR)** — US tens to low hundreds of thousands; Global similar age/ancestry patterns.
10. **Mixed connective tissue disease / overlap syndromes** — Variable; overlaps common.
11. **ANCA-associated vasculitis** (GPA, MPA, EGPA) — US/ Global tens of thousands each; collective higher.
12. **Behçet's disease** — US low tens of thousands; Global higher in Silk Road regions.
13. **Adult-onset Still's disease** — Rare; US/ Global thousands.
14. **Relapsing polychondritis** — Very rare.
15. **Sarcoidosis** (with rheumatologic features; sometimes managed by rheum) — US higher prevalence; Global variable.

Lower Tier (Rarer but Important in Practice)

16–25. Include specific or related entities such as:

- **Immune-mediated necrotizing myopathy** (subset of myositis)
- **Antisynthetase syndrome**
- **Undifferentiated connective tissue disease**
- **Juvenile idiopathic arthritis (adult care transition)**
- **IgG4-related disease** (multisystem, often rheum-managed)
- **Cryoglobulinemic vasculitis**
- **Hypocomplementemic urticarial vasculitis**
- **Primary angiitis of the CNS** (rare)
- **Takayasu arteritis**
- **Polyarteritis nodosa** or other medium-vessel vasculitis